

The 8% Payer Underpayment Leak

Why paying claims != paying contracted rates.

The Reality: Practices lose 7% to 11% of top-line revenue because payers underpay contracted fee schedules silently.

The Anatomy of an Underpayment

Silent reimbursement shrinkage on commercial remittances.

- **Outdated Fee Tables:** Payer adjudicates claims against last year's conversion factor.
- **Silent Downcoding:** 99214 remitted at 99213 reimbursement without denial notification.
- **Lesser-Of Clauses:** Payer reimburses lower billed charge instead of higher contract allowables.

Why Manual Billing Teams Miss It

Human billers only look for bash denial codes.

- **Remit Blindspot:** If an EDI 835 shows payment, staff posts and closes the line item.
- **No Contract Audit:** Few billing teams compare line-item allowables to written contracts.
- **Cumulative Loss:** 0 underpayment across 200 monthly visits = 6,000 lost per year.

Algorithmic Fee Schedule Auditing

Automating contract compliance on every single remittance.

- **Digitized Fee Schedules:** Every payer contract allowable uploaded into rule engine.
- **Instant Variance Flags:** Remittances differing from contracted rates flagged automatically.
- **Bulk Underpayment Recovery:** Grouped appeal batches submitted within 30 days.

Audit Your Contract Allowables

Find out how much contracted revenue your payers withheld this quarter.

Run an Underpayment Audit: Benchmark your contracts at aetherahealthcare.com/free-assessment